

BURTON HOSPITALS NHS FOUNDATION TRUST
REFERRAL FORM FOR PATIENTS WITH
SUSPECTED CANCER

HOSPITAL USE Date Fax Received

Consultant

Appointment Date Time

Patient Information

Send via e-Referral Service

Patient NHS No: **Not known**

Surname: **Mickey**

Forename: **Mouse**

Address: **St Chads**
Cannock
WS7 0EW

Date of Birth: **10-Feb-2000**

Age: **26y**

Hospital No

Patient's Daytime Telephone No:

Patient's Evening Telephone No:

Patient's Mobile Telephone No:

Patient is happy for a message to be left? YES ☐ NO ☐

Patient gender: **Male**

Does the patient know that cancer is a possibility?

YES ☐ NO ☐

Does the patient have any special needs? YES ☐ NO ☐

- Key Relative/Carer

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-Ambulance transport required?

YES ☐ NO ☐

Sensory deficit? YES ☐ NO ☐

If yes please specify

Interpreter (specify)

- Physical impairment (detail)

-Is the patient an overseas visitor? YES ☐ NO ☐

Has the patient previously visited this hospital? YES ☐ NO ☐

Referring GP: **n**

PCG Code:

Registered GP: **WARD, James (Dr)**

GP Tel No: 01543 682654

GP FAX NO.: **N/A- via email:**
darwinmedical.practice@nhs.net

Practice Stamp:

Darwin Medical Practice
Greenwood Health Centre
Lichfield Road
Burntwood
Staffordshire
WS7 0AQ
Tel: 01543 682654
Email: darwinmedical.practice@nhs.net

Date of Decision to Refer: **7 May 2026**

Signature of GP: **n**

Tumour Site	1. Lung ☐	4. Breast (use Derby form) ☐	7. Haematological ☒	10. Brain/CNS (DO NOT USE) ☐
(Please Specify)	2. Upper GI ☐	5. Gynaecological ☐	8. Skin ☐	11. Sarcomas (Use Derby Hospital) ☐
	3. Lower GI ☐	6. Urological ☐	9. Head & Neck ☐	

I confirm that the patient fulfils the following criteria for urgent referral for suspected cancer (use coding chart)

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Clinical examination/investigations undertaken:

Information:- (please provide relevant and concise medical history and co-morbidities)
PMH and current medication list is attached with referral.

For hospital use:

Date received _____ Time received _____ Date 1st appt booked _____ Date of 1st appt _____

Form completed adequately? Yes ☐ No ☐ Referral criteria appropriately applied? Yes ☐ No ☐

Coding Chart

1. LUNG CANCER - Chest x ray suggestive of lung cancer (including pleural effusion and slowly resolving consolidation). - Persistent haemoptysis in smokers/ex-smokers over 40 years of age. - High suspicion of lung cancer with normal x-ray. - History if asbestos exposure, recent onset chest pain, shortness of breath, or where chest x-ray indicates pleural effusion, pleural mass or any suspicious lung pathology. <i>For other lung symptoms consider urgent chest x-ray first.</i> REFER IMMEDIATELY PATIENTS WITH - Signs of superior vena cava obstruction (swelling of the face/neck with fixed elevation of jugular venous pressure). - Stridor.	CODE 1 A 1B 1C 1D	3. LOWER G.I. CANCER All ages - A definite palpable right-sided lower abdominal mass. - A definite palpable rectal (not pelvic) mass. Aged 40 and over - Rectal bleeding WITH a change in bowel habit to looser stools and/or increased frequency persistent for >=6 weeks. Over 60 years - Rectal bleeding persistently WITHOUT anal symptoms or change in bowel habit. - Change of bowel habit to looser stools and/or increased frequency WITHOUT rectal bleeding and persistent for >=6 weeks. Any age - Iron deficiency anaemia WITHOUT an Obvious cause (Hb <11g/100ml in men or <10g/100ml in postmenopausal women. Please ensure that a full blood count, abdominal or rectal examination has been carried out as appropriate prior to referral.	CODE 3A 3B 3C 3D 3E 3F	5. GYNAECOLOGY CANCER - Clinical features suggestive of cervical cancer on examination. - Vulval bleeding due to ulceration or an unexplained vulval lump. - Palpable pelvic or abdominal mass not obviously fibroids or of gastrointestinal or urological origin. - Postmenopausal bleeding where patient not on hormone therapy, or where patient taking tamoxifen. - Persistent intermenstrual bleeding and negative pelvic examination. - On HRT with unexpected or prolonged bleeding persisting for more than 6 weeks after stopping HRT.	CODE 5A 5B 5C/D 5E 5F 5G
2. UPPER G.I. CANCER - Dysphagia – food sticking on swallowing (any age). - Dyspepsia at any age combined with one or more of the following ‘alarm’ symptoms - weight loss - proven anaemia - vomiting - epigastric mass - dysphagia - chronic gastrointestinal bleeding - suspicious barium meal result - Unexplained persistent recent onset dyspepsia in a patient aged 55 years or more. - Obstructive jaundice. - Upper abdominal mass without dyspepsia. - Unexplained upper abdominal pain and weight loss, with or without back pain. CONSIDER urgent referral for patients with: - Unexplained worsening of dyspepsia combined with: - Barrett’s oesophagus - Peptic ulcer surgery over 20 years ago - Known dysplasia, atrophic gastritis - Intestinal metaplasia. - Persistent vomiting and weight loss in the absence of dyspepsia. - Unexplained weight loss or iron deficiency anaemia in the absence of dyspepsia.	2A 2B 2C 2D 2E 2F 2G 2H 2I	4. BREAST CANCER - Female patients with a lump: - any age – with a discrete, hard lump <u>with fixation</u>, with or without skin tethering - aged >=30 years – with a discrete lump that persists after their next period or presents after menopause. - Aged <30 years – with a lump that enlarges, is fixed and hard, or in whom other reasons for concern apply e.g. family history. - Previous breast cancer patients – with a further lump or suspicious symptoms. - Patients with one of the following nipple problems: - unilateral eczematous skin or nipple change that does not respond to topical treatment. - nipple distortion of recent onset. - spontaneous unilateral bloody nipple discharge. - Male patients, aged >=50 years with a Unilateral, firm subareolar mass with or without nipples distortion or associated skin changes.	4A 4B 4C 4D	6. UROLOGICAL CANCER - Macroscopic Haematuria in adults. - Microscopic Haematuria in adults over 50 years. - Swelling or mass in the body of the testis. - Abdominal mass identified clinically or on imaging, thought to arise from urinary tract. - Recurrent or persistent urinary tract infection associated with haematuria in adults aged >=40 years. - An elevated age specific PSA in men who are not significantly compromised by other co-morbidities. - A high PSA (>20ng/ml) in men with a clinically malignant prostate. (PSA result should accompany referral) - Any suspected penile cancer. - With symptoms and high PSA levels.	6A 6B 6C 6D 6E 6F 6G 6H 6I

Coding Chart

7. HAEMATOLOGICAL CANCERS • Constellation of 3 or more of the following symptoms: fatigue, night sweats, weight loss, itching, breathlessness, bruising, recurrent infections, bone pain, fever, bleeding, alcohol-induced pain, abdominal pain, lymphadenopathy, splenomegaly. • Persistent unexplained splenomegaly. REFER IMMEDIATELY • Patients with a blood count/film reported as acute leukaemia. • Patients with special cord compression or renal failure suspected of being caused by myeloma.	CODE 7A 7B	9. HEAD AND NECK CANCERS Urgent Referral • An unexpected persistent sore or painful throat. • Unilateral unexplained pain in the hand and neck area >4 weeks associated with otalgia but normal otoscopy. • Unexplained ulceration of oral mucosa or mass persisting > 3 weeks. • All red or red and white patches of the oral mucosa that are painful and swollen/bleeding. • Unexplained neck lump of recent onset or previous undiagnosed lump changing over 3-6 weeks. • Unexplained persistent swelling in parotid or submandibular gland. • Thyroid swelling associated with: - A solitary nodule increasing in size. - A history of neck irradiation. - Family history of endocrine tumour. - Unexplained hoarseness or voice changes. - Cervical lymphadenopathy. - Pre-pubertal patient. - Patient aged >= 65 years. REFER IMMEDIATELY • Patients with symptoms of tracheal compression including stridor due to thyroid swelling.	Refer to ENT ENT/OMF OMF OMF ENT/OMF ENT/OMF ENT	CODE 9A 9B 9C 9D 9E 9F 9G	10. BRAIN TUMOURS • Progressive neurological deficit developing over days to weeks (e.g. weakness, sensory loss, dysphasia, ataxia). • New or recent onset seizures. • Headaches or mental changes. • Cranial nerve palsy, unilateral sensorineural deafness • Headaches of recent onset accompanied by features if raised intracranial pressure eg vomiting, drowsiness, post-related headache, pulse-synchronous tinnitus, blackout, change in personality or memory, a new qualitatively different/unexplained headache that becomes progressively severe. Consider urgent referral for: • Rapid progression of subacute focal neurological deficit, unexplained cognitive impairment, behavioural disturbance or slowness, personality changes confirmed by a witness for which there is no reasonable explanation.	CODE 10A 10B 10C 10D 10E 10F									
8. SKIN CANCERS NB – EXCISION IN PRIMARY CARE SHOULD BE AVOIDED Melanoma • With a lesion suspicious of melanoma, that is changing and/or scoring 3 points or more from the features below (based on major features scoring two points each and minor features scoring one point each) :-<table><tr><td>Major Features</td><td>Minor Features</td></tr><tr><td>Change in size</td><td>Large diameter =>7mm</td></tr><tr><td>Irregular shape</td><td>Inflammation</td></tr><tr><td>Irregular colour</td><td>Oozing</td></tr><tr><td></td><td>Change in sensation</td></tr></table> • With unexpected histological diagnosis of Melanoma. SCC • Non-healing keratinising or crusted tumours >1cm with significant induration on palpation and documented expansion over 8 weeks. • With unexpected histological diagnosis of squamous cell carcinoma. • Who have had an organ transplant with new/growing cutaneous lesions. Visit www.sunsmart.org.uk for pictorial guidance.	Major Features	Minor Features	Change in size	Large diameter =>7mm	Irregular shape	Inflammation	Irregular colour	Oozing		Change in sensation	8Ai 8Aii 8Bi 8Bii 8C				
Major Features	Minor Features														
Change in size	Large diameter =>7mm														
Irregular shape	Inflammation														
Irregular colour	Oozing														
	Change in sensation														
11. BONE CANCER AND SARCOMAS • A palpable lump that is: - Size > 5 cms. - Painful. - Increasing in size. - Deep to fascia, fixed or immobile. - Recurrence after previous excision. • Patients with radiological suspicion of a bone cancer. REFER FOR IMMEDIATE X-RAY PATIENTS WITH SUSPECTED SPONTANEOUS FRACTURE.						11A 11A1 11A2 11A3 11A4 11A5 11B									

